PEDIATRIC SURGERY · LECTURE 1 · QUESTION BANK

Pediatric Surgical Care

الرعاية الجراحية للأطفال — بنك الأسئلة

10	6	12
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

أسئلة سابقة · فاينال (مراجعة أسئلة) · ميدترم السنة اللي فاتت (Microsoft Forms)

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 Bilious vomiting in neonates: — ميدترم السنة اللي فاتت (Microsoft Forms)

a. Is an emergency
b. Surgical consultation is a must
c. It may be life threatening
d. All of the above

2 You should teach the parents, when needed, all of the following EXCEPT: — ميدترم السنة اللي فاتت (Microsoft Forms)

a. Colostomy care
b. Gastrostomy feeds
c. Esophagostomy care
d. Intravenous antibiotics

3 Complete: A complication of blood transfusion is ..... — فاينال (مراجعة أسئلة)

.....

4 Complete: Complications of laparoscopic surgery are ..... — فاينال (مراجعة أسئلة)

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5 Complete: Signs of severe hypothermia ..... — فاينال (مراجعة أسئلة)

.....

6 Complete: Hypothermia in the neonate may ..... — فاينال (مراجعة أسئلة)

.....

7 Central venous access needs special handling and frequent dressing. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

8 You should ask the pediatric surgeon before you start feeds in gastrostomy. (True / False) فاينال (مراجعة أسئلة)

a. True **b.** False

9 You should not teach parents colostomy care. (True / False) فاينال (مراجعة أسئلة)

a. True **b.** False

10 Bilious vomiting can be an emergency. (True / False) أسئلة سابقة

a. True **b.** False

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 Which pain scale is suitable for a 2-year-old after surgery? NCLEX

a. Numeric 0–10 scale **b.** FLACC scale
c. Visual analogue scale **d.** McGill questionnaire

N2 Parents refuse a life-saving emergency operation for their child. The surgeon proceeds. Which principle justifies this? NCLEX

a. Autonomy of parents
b. Best interest of the child (beneficence)
c. Fidelity
d. Veracity

N3 Select ALL components of preoperative care of a child. SELECT ALL

a. Hydration and NPO status
b. Age-appropriate preparation to reduce anxiety
c. Informed consent
d. Monitoring vital signs
e. Giving a heavy meal before anesthesia

N4 Why are neonates at special risk of hypothermia in the operating room? NCLEX

a. Thick subcutaneous fat
b. Large surface area to weight ratio and little fat
c. They shiver efficiently
d. Low metabolic rate

N5 A child receiving multiple blood transfusions is at risk of: NCLEX

a. Hypokalemia **b.** Hemosiderosis (iron overload)
c. Hypoglycemia **d.** Polycythemia

N6 **A neonate with bilious vomiting is admitted. The nurse's priority is:** NCLEX

a. Give oral fluids

b. Keep NPO, insert NGT, start IV fluids and notify the surgeon

c. Wait and observe

d. Give antiemetic and feed

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Pediatric surgery differences	Anatomical, physiological and psychological.	**Informed consent**	From parents/guardian; child assent according to age.
Emergency without consent	Allowed for child's best interest.	**Pre-op care**	NPO, hydration, emotional support, pain management, monitoring.
Post-op assessment	Vital signs, pain (FLACC/Wong-Baker), wound, neurologic status.	**Bilious vomiting**	Surgical emergency in neonate until proven otherwise.
Hypothermia risk	Large surface area, little fat, exposure in OR.	**Hypothermia management**	Warm environment, remove wet clothes, blankets, warm fluids, warmed O2.
Blood transfusion complications	Hemolytic reaction, overload, hyperkalemia, embolism, thrombophlebitis, hemosiderosis.	**Laparoscopy pros**	Minimally invasive, faster recovery, cosmetic, less blood loss.
Central venous line	Special handling and frequent sterile dressing.	**Gastrostomy feeds**	Start after surgeon's order; teach parents.

Answer Key

الإجابات

1. d — All of the above

2. d — Intravenous antibiotics

3. ✎ — Hyperkalemia (also hemolytic reaction, circulatory overload, embolism, thrombophlebitis, hemosiderosis)

4. ✎ — Vessel injury, organ injury, bowel injury

5. ✎ — Loss of shivering, hypotension, hypoactivity/coma, cardiac arrest

6. ✎ — Cause death

7. a — True

8. a — True

9. b — False

10. a — True

N1. b — FLACC scale
FLACC (Face, Legs, Activity, Cry, Consolability) suits preverbal children; Wong-Baker FACES for ~3 years and older.

N2. b — Best interest of the child (beneficence)
In emergencies the child's best interest allows treatment without consent.

N3. a, b, c, d — Hydration and NPO status; Age-appropriate preparation to reduce anxiety; Informed consent; Monitoring vital signs
Preoperative care: preparation (NPO, hydration), emotional support, consent, pain management and monitoring.

N4. b — Large surface area to weight ratio and little fat
Large surface area, thin skin and limited fat increase heat loss; shivering is absent in neonates.

N5. b — Hemosiderosis (iron overload)
Repeated transfusions lead to iron overload; hyperkalemia is another complication.

N6. b — Keep NPO, insert NGT, start IV fluids and notify the surgeon
Bilious vomiting in a neonate is a surgical emergency (e.g., malrotation/volvulus) until proven otherwise.